

*Every task is anchored to UNREAD TIME in Sectra unless noted (e.g., CT hyperacute stroke uses exam start time).

*The SHIFT TIME column shows when the rotation is on-shift; the UNREAD TIME column shows the case window each task covers.

WEEKDAY EVENING SHIFTS					
Rotation / Shift Time	Day	TASK	UNREAD TIME	WORKLIST	COMMENTS
Evening CVH 16:00 – 21:59	Weekday	Plain films ED/IP CVH	15:46 – 19:59	CVH ACI X-Ray	Any interpretations requested before 22:00 (final report).
		Plain films OP (CVH or QHC incl. UCC).	-	CVH/MH OP Xray / QHC Xray and ACI CT	16:00–22:00: only if contacted by tech/referring MD.
		Plain film consults — QHC IP.	-	QHC Xray and ACI CT	16:00–22:00: only if contacted by tech/referring MD.
		CT (all other, CVH/QHC): incl. CTA-H/N; excl. hyperacute stroke, peripheral run-offs, VIR-covered CT (see VIR notes).	15:46 – 21:59	CVH ACI EVENING/WEEKEND / CT/US	• IP CT (except brains), unread >19:00 → call rad = prelim (acute; address non-acute on request). Final = next-day ACI-AM/weekend rad. • ED/IP HRL → call rad = prelim. Final = next Thursday's HRL rad. • Urgent OP CT: included; tag to ACI worklist.
		CT HYPERACUTE STROKE	16:00 – 21:59 (start not unread time)	CVH ACI EVENING/WEEKEND / CT/US	Call rad responsible if exam starts before shift end, unless handed off rad-to-rad.
		CT peripheral run-offs; VIR-covered CT (see VIR notes on endograft patients).	15:46 – 21:59	CVH EVENING/WEEKEND / CT/US	Call rad = prelim (acute). Final = VIR rad. VIR rad available for backup (no trade back).
		US - ED/IP (CVH)	15:46 – 21:59	CVH EVENING/WEEKEND / CT/US	
		MRI – all IP/ER	14:30 – 21:59	THP ED/IP MRI	Only if referring MD requests. Prelim = on-call rad if requested; if on-call rad doesn't do MRI → other-site rad or backup rad (trade). Final = next-day THP MR ACI or weekend day rad.
		Nuclear Medicine	16:00 – 21:59		1st call for questions. Trade back with NM rad for urgent studies completed after 16:00 (see NM section).
		Protocol ED/IP CT + VIR CT; consults from techs/referring clinicians.	16:00 – 21:59		May include contrast issues → if physician assessment needed, send to ED.
Evening MH 16:00 – 21:59	Weekday	Plain films ED/IP MH	15:46 – 19:59	MH ACI X-Ray	Any interpretations requested before 22:00 (final report).
		Plain films (urgent OP).	-	CVH/MH OP Xray	16:00–22:00: only if contacted by tech/referring MD and/or tagged to you or ACI list.
		CT HYPERACUTE STROKE	16:00 – 21:59 (start not unread time)	MH ACI EVENING/WEEKEND / CT/US	Call rad responsible if exam starts before shift end, unless handed off rad-to-rad.
		CT (all other, MH incl. UCC): incl. CTA-H/N; excl. hyperacute stroke, peripheral run-offs, VIR-covered CT (see VIR notes).	15:46 – 21:59	MH ACI EVENING/WEEKEND / CT/US	• IP CT (except brains), unread >19:00 → call rad = prelim (acute; address non-acute on request). Final = next-day ACI-AM/weekend rad. • ED/IP HRL → call rad = prelim. Final = next Thursday's HRL rad. • Urgent OP CT: included; tag to ACI worklist.
		CT peripheral run-offs; VIR-covered CT (see VIR notes on endograft patients).	15:46 – 21:59	MH ACI EVENING/WEEKEND / CT/US	Call rad = prelim (acute). Final = VIR rad. VIR rad available for backup (no trade back).
		US ED/IP (MH); US urgent OP QHC (tagged ACI, incl. UCC).	15:46 – 21:59	MH ACI EVENING/WEEKEND / CT/US	Incl. consults on OP US from QHC.
		MRI – all IP/ER	14:30 – 21:59	THP ED/IP MRI	Only if referring MD requests. Prelim = on-call rad if requested; if on-call rad doesn't do MRI → other-site rad or backup rad (trade). Final = next-day THP MR ACI or weekend day rad.
		Nuclear Medicine	16:00 – 21:59		1st call for questions. Trade back with NM rad for urgent studies completed after 16:00 (see NM section).
		Protocol ED/IP CT + VIR CT; consults from techs/referring clinicians.	16:00 – 21:59		May include contrast issues → if physician assessment needed, send to ED.
VIR Weekday 16:00 – 07:59	Weekday	VIR Procedures	16:00 – 07:59		Requires direct physician-to-rad consult. MRT and VIR nursing on call via locating.
		Procedures — US or FL-guided.	16:00 – 07:59		Requires direct physician-to-rad consult. Urgent US or FL-guided procedures at any site, even if done by a general rad during regular workday (e.g. LP, joint aspiration).
		CTA run-off; CTA on aortic-endograft patients (EVAR/TEVAR).	16:00 – 07:59	THP VIR CT	Prelim = diagnostic rad. Final = VIR rad, by 09:30 the next calendar day (next-day VIR at study site; weekend/stat → next-day on-call VIR). Endograft CT: VIR only if indication relates to aorta/graft. Body CTA without endograft = diagnostic rad, regardless of tagging.
		MR backup: if neither the on-shift rad (both sites) nor the backup rad reads MRI.	16:00 – 07:59		
Nuclear Medicine 16:00 – 20:00	Weekday	Daytime urgent NM studies not completed by 16:00.	16:00 – 20:00		General rad = 1st call for NM questions. No evening NM tech coverage. Urgent case after 16:00 → NM rad may trade back an enhanced OP case to late-shift rad (VQ ↔ CT Chest; GI bleed ↔ CT AP; WBC ↔ Spine/MSK CT). Non-urgent IP → next-day NM rad.
Weekday Evening + Overnight Backup 16:00 – 07:59	Weekday	Available within 45 min of page. • Initiated by working rad, chief, or delegate. • Cases <22:00: 1:1 future OP-case trade (similar intensity). • Cases ≥22:00 (overnight): 2:1 future OP-case trade. • Non-MR rad requests MRI backup after 22:00 → backup rad also gets a future evening backup shift from the requester (in addition to the case trade).			

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WEEKEND DAYTIME SHIFTS					
Rotation / Shift Time	Day	TASK	UNREAD TIME	WORKLIST	COMMENTS
Weekend CVH Daytime 08:00 – 15:59	Sat/Sun/Stat	Plain films ED/IP CVH	08:00 – 15:45	CVH ACI X-Ray	
		Plain films OP (CVH or QHC).	-	CVH/MH OP Xray / QHC Xray and ACI CT	08:00–15:59: only if contacted by tech/referring MD and/or tagged to ACI or rad.
		Plain film consults — QHC IP.	-	QHC Xray and ACI CT	08:00–15:59: only if contacted by tech/referring MD.
		ER CT Head – CVH site	22:00 (day before) – 07:59	CVH ACI EVENING/WEEKEND / CT/US	Exclude prior-day/overnight cases with prelim from another rad. Includes unreported IP CT brains from previous night.
		IP CT at CVH/QHC (except unenhanced CT Brain) — prelim from on-call rad.	19:00 (day before) – 07:59	CVH ACI EVENING/WEEKEND / CT/US	Final = weekday daytime rad. Prelims = evening/overnight/early rads.
		CT (all other, CVH/QHC): incl. CTA-H/N; excl. hyperacute stroke, peripheral run-offs, VIR-covered CT (see VIR notes).	08:00 – 15:59	CVH ACI EVENING/WEEKEND / CT/US	- Urgent OP CT (mainly PE): included; tag to ACI list. - ED/IP HRL → call rad = prelim. Final = next Thursday's HRL rad.
		CT HYPERACUTE STROKE	08:00 – 15:59 (start not unread time)	CVH ACI EVENING/WEEKEND / CT/US	Call rad responsible if exam starts before shift end (unread time >16:00), unless handed off verbally rad-to-rad.
		CT peripheral run-offs; VIR-covered CT (see VIR notes on endograft patients).	08:00 – 15:59	CVH ACI EVENING/WEEKEND / CT/US	Call rad = prelim (acute). Final = VIR rad. VIR rad available for backup (no trade back).
		US - ED/IP (CVH)	08:00 – 15:59	CVH ACI EVENING/WEEKEND / CT/US	
		MRI – all IP/ER	CVH cases: 14:30 (day before) – 9:59 (same day)	THP ED/IP MRI	Cases in the assigned window require final reports; if providing prelims in the assigned window, final reports = latest by 4PM Mon (or Tues after a long weekend). For cases outside the assigned window: on-call rad if referring MD requests a report; if on-call rad doesn't do MRI → other-site rad or backup rad (trade). Final = rad on subsequent shift per unread time.
		Nuclear Medicine	08:00 – 15:59		1st call for questions. No weekend NM tech coverage.
		Protocol ED/IP CT + VIR; consults from techs/referring clinicians.	08:00 – 15:59		May include contrast issues → if physician assessment needed, send to ED.
Weekend MH Daytime 08:00 – 16:00	Sat/Sun/Stat	Evening Radiologist Backup	16:00 – 21:59		Available within 45 min of page for the CVH evening rad. - Initiated by working rad or on-call chief. - Cases <22:00: 1:1 future OP-case trade (similar intensity).
		Plain films ED/IP M	08:00 – 15:45	MH ACI X-Ray	
		Plain film consults — OP.	-	CVH/MH OP Xray	08:00–16:00: only if contacted by tech/referring MD and/or tagged to ACI.
		ER/IP CT Head (MH site) — incl. UCC cases from previous night.	22:00 (day before) – 07:59	MH ACI EVENING/WEEKEND / CT/US	Exclude prior-day/overnight cases with prelim from another rad. Includes unreported IP CT brains from previous night.
		IP CT at MH (except unenhanced CT Brain) — prelim from on-call rad.	19:00 (day before) – 07:59	MH ACI EVENING/WEEKEND / CT/US	Final = weekday daytime rad. Prelims = evening/overnight/early rads.
		CT (all other, MH): incl. CTA-H/N; excl. hyperacute stroke, peripheral run-offs, VIR-covered CT (see VIR notes).	08:00 – 15:59	MH ACI EVENING/WEEKEND / CT/US	- Urgent OP CT (mainly PE, incl. rad referrals): included. - ED/IP HRL → call rad = prelim. Final = next Thursday's HRL rad.
		CT HYPERACUTE STROKE	08:00 – 15:59 (start not unread time)	MH ACI EVENING/WEEKEND / CT/US	Call rad responsible if exam starts before shift end (unread time >16:00), unless handed off rad-to-rad.
		CT peripheral run-offs; VIR-covered CT (see VIR notes on endograft patients).	08:00 – 15:59	MH ACI EVENING/WEEKEND / CT/US	Call rad = prelim (acute). Final = VIR rad. VIR rad available for backup (no trade back).
		US ED/IP (MH); US urgent OP QHC (tagged to ACI, incl. UCC).	08:00 – 15:59	MH ACI EVENING/WEEKEND / CT/US	Incl. consults on OP US from QHC.
		MRI – all IP/ER	MH cases: 14:30 (day before) – 9:59 (same day)	THP ED/IP MRI	Cases in the assigned window require final reports; if providing prelims in the assigned window, final reports = latest by 4PM Mon (or Tues after a long weekend). For cases outside the assigned window: on-call rad if referring MD requests a report; if on-call rad doesn't do MRI → other-site rad or backup rad (trade). Final = rad on subsequent shift per unread time.
		Nuclear Medicine	08:00 – 15:59		1st call for questions. No weekend NM tech coverage.
		Protocol ED/IP CT; consults from techs/referring clinicians.	08:00 – 15:59		May include contrast issues → if physician assessment needed, send to ED.
		Evening Radiologist Backup	16:00 – 21:59		Available within 45 min of page for the MH evening rad. - Initiated by working rad or on-call chief. - Cases <22:00: 1:1 future OP-case trade (similar intensity).

		THP RADIOLOGY ON CALL		VERSION: July 10, 2026		PAGE 3/4	
		REPORTING RESPONSIBILITIES					
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WEEKEND EVENING SHIFTS							
Rotation / Shift Time	Day	TASK	UNREAD TIME	WORKLIST	COMMENTS		
Weekend CVH Evening 16:00 – 21:59	Sat/Sun/Stat	Plain films ED/IP CVH	15:46 – 19:59	CVH ACI X-Ray	20:00–21:59: only if contacted by tech/referring MD.		
		Plain films OP (CVH or QHC).	-	CVH/MH OP Xray / QHC Xray and ACI CT	16:00–21:59: only if contacted by tech/referring MD.		
		Plain film consults — QHC IP.	-	QHC Xray and ACI CT	16:00–21:59: only if contacted by tech/referring MD.		
		US - ED/IP (CVH)	16:00 – 21:59	CVH ACI EVENING/WEEKEND / CT/US			
		CT HYPERACUTE STROKE (CVH/QHC)	16:00 – 21:59 (start not unread time)	CVH ACI EVENING/WEEKEND / CT/US	Call rad responsible if exam starts before shift end (unread time >22:00), unless handed off verbally rad-to-rad.		
		CT (all other, CVH/QHC incl. UCC): incl. CTA-H/N; excl. hyperacute stroke, peripheral run-offs, VIR-covered CT (see VIR notes).	16:00 – 21:59	CVH ACI EVENING/WEEKEND / CT/US	• IP CT (except brains), unread >19:00 → call rad = prelim (acute; address non-acute on request). Final = next-day ACI-AM/weekend rad. • ED/IP HRL → call rad = prelim. Final = next Thursday's HRL rad. • Urgent OP CT: included; tag to ACI worklist.		
		CT peripheral run-offs; VIR-covered CT (see VIR notes on endograft patients).	16:00 – 21:59	CVH ACI EVENING/WEEKEND / CT/US	Call rad = prelim (acute). Final = VIR rad. VIR rad available for backup (no trade back).		
		MRI – all IP/ER	CVH cases: 10:00 – 14:29 (same day)	THP ED/IP MRI	Cases in the assigned window require final reports; if providing prelims in the assigned window, final reports = latest by 4PM Mon (or Tues after a long weekend). For cases outside the assigned window: on-call rad if referring MD requests a report; if on-call rad doesn't do MRI → other-site rad or backup rad (trade). Final = rad on subsequent shift per unread time.		
		Nuclear Medicine	16:00 – 21:59		1st call for questions. No weekend NM tech coverage.		
		Protocol ED/IP CT + VIR CT; consults from techs/referring clinicians.	16:00 – 21:59		May include contrast issues → if physician assessment needed, send to ED.		
Daytime Radiologist Backup	08:00 – 15:59		Available within 45 min of page for the CVH daytime rad. • Initiated by working rad or on-call chief. • Cases <22:00: 1:1 future OP-case trade (similar intensity).				
Weekend MH Evening 16:00 – 22:00	Sat/Sun/Stat	Plain films ED/IP M	15:46 – 19:59	MH ACI X-Ray	20:00–21:59: only if contacted by tech/referring MD.		
		Plain film consults — OP.	-	CVH/MH OP Xray	16:00–21:59: only if contacted by tech/referring MD.		
		CT (all other, MH): incl. CTA-H/N; excl. hyperacute stroke, peripheral run-offs, VIR-covered CT (see VIR notes).	16:00 – 21:59	MH ACI EVENING/WEEKEND / CT/US	• IP CT (except brains), unread >19:00 → call rad = prelim (acute; address non-acute on request). Final = next-day ACI-AM/weekend rad. • ED/IP HRL → call rad = prelim. Final = next Thursday's HRL rad. • Urgent OP CT: included; tag to ACI worklist.		
		CT HYPERACUTE STROKE	16:00 – 21:59 (start not unread time)	MH ACI EVENING/WEEKEND / CT/US	Call rad responsible if exam starts before shift end (unread time >22:00), unless handed off verbally rad-to-rad.		
		CT peripheral run-offs; VIR-covered CT (see VIR notes on endograft patients).	16:00 – 21:59	MH ACI EVENING/WEEKEND / CT/US	Call rad = prelim (acute). Final = VIR rad. VIR rad available for backup (no trade back).		
		US ED/IP (MH); US urgent OP QHC (tagged ACI, incl. UCC).	16:00 – 21:59	MH ACI EVENING/WEEKEND / CT/US	Incl. consults on OP US from QHC.		
		MRI – all IP/ER	MH cases: 10:00 – 14:29 (same day)	THP ED/IP MRI	Cases in the assigned window require final reports; if providing prelims in the assigned window, final reports = latest by 4PM Mon (or Tues after a long weekend). For cases outside the assigned window: on-call rad if referring MD requests a report; if on-call rad doesn't do MRI → other-site rad or backup rad (trade). Final = rad on subsequent shift per unread time.		
		Nuclear Medicine	16:00 – 21:59		1st call for questions. No weekend NM tech coverage.		
		Protocol ED/IP CT; consults from techs/referring clinicians.	16:00 – 21:59				
		Daytime Radiologist Backup	08:00 – 15:59		Available within 45 min of page for the MH daytime rad. • Initiated by working rad or on-call chief. • Cases <22:00: 1:1 future OP-case trade (similar intensity).		
VIR Weekend 08:00 – 08:00	Fri 16:00 - Mon 07:59	Plain films ED/IP M and C	20:00 (Previous day) – 07:59 (Same Day)	THP ACI XRAY	Urgent interpretations = daytime or evening rad at the site.		
		Plain films UCC and IP Q	Sat rad: Fri 15:00 – Sat 07:59. Sun rad: Sat 08:00 – Sun 07:59.	THP ACI XRAY	Urgent interpretations = daytime or evening rad at C site.		
		VIR Procedures	08:00 – 07:59		Requires direct physician-to-rad consult. MRT and VIR nursing on call via locating.		
		Procedures — US or FL-guided.	08:00 – 07:59		Requires direct physician-to-rad consult. Urgent US or FL-guided procedures at any site, even if done by a general rad during regular workday (e.g. LP, joint aspiration).		
		CTA run-off; CTA on aortic-endograft patients (EVAR/TEVAR).	08:00 – 07:59	THP VIR CT	Prelim = diagnostic rad. Final = VIR rad, by 09:30 the next calendar day (next-day VIR at study site; weekend/stat → next-day on-call VIR). Endograft CT: VIR only if indication relates to aorta/graft. Body CTA without endograft = diagnostic rad, regardless of tagging.		
		MR backup: if neither the on-shift rad (both sites) nor the backup rad reads MRI.	08:00 – 07:59				

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OVERNIGHT SHIFTS					
Rotation / Shift Time	Day	TASK	UNREAD TIME	WORKLIST	COMMENTS
General Overnight / Early AM 22:00 – 07:59	Every overnight	CT Head consults – ED (M/C/Q)	-	THP Overnight ACI CT + US	Only if referring clinician calls between 22:00 and 07:59 (independent of unread time). Finals expected.
		CT (all other, C/M/Q): incl. CTA-H/N + hyperacute stroke + IP CT unenhanced Head CVH/MH (unless ordering MD is a THP neurosurgeon/neurologist). Excl. peripheral run-offs, T/EVAR aortic pathology (see VIR notes).	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)	THP Overnight ACI CT + US	• IP CT (except brains) → call rad = prelim (acute; address non-acute on request). Final = next-day ACI-AM/weekend rad. • ED/IP HRL → call rad = prelim. Final = next Thursday's HRL rad. • Studies with unread time after 07:00 → next-day rad, unless referring MD contacts before 07:59 (then overnight/early-AM rad = final). • Exception: hyperacute stroke — call rad responsible if exam starts before shift end (unread time >08:00), unless handed off rad-to-rad.
		CT peripheral run-offs; VIR-covered CT (see VIR notes on endograft patients).	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)	THP Overnight ACI CT + US	Call rad = prelim (acute). Final = VIR rad. VIR rad available for backup (no trade back).
		US - ED/IP (M/C/QHC)	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)	THP Overnight ACI CT + US	Studies with unread time after 07:00 → next-day rad, unless referring MD contacts before 07:59 (then overnight/early-AM rad = final).
		Plain film consults — any site, incl. urgent OP.	-	THP ACI XRAY	Any case if contacted by referring clinician/tech between 22:00 and 07:59 (finals expected).
		MRI – all IP/ER	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)	THP ED/IP MRI	Only if referring MD requests. Prelim = on-call rad if requested; if on-call rad doesn't do MRI → other-site rad or backup rad (trade). Final = next-day THP MR ACI or weekend day rad.
		Nuclear Medicine	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)		1st call for questions. No overnight NM tech coverage.
		Protocol ED/IP CT + VIR; consults from techs/referring clinicians.	22:00 – 03:59 (Overnight) 04:00 – 07:59 (Early)		
Weekend Overnight Backup 22:00 – 07:59	Every overnight	• Available within 45 min of page. ◦ Initiated by working rad, chief, or delegate. ◦ Overnight cases (≥22:00): 2:1 future OP-case trade (similar intensity). • Non-MR rad requests MRI backup after 22:00 → backup rad also gets a future evening backup shift from the requester (in addition to the case trade).			